

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase III, Randomised, Open-label, Global Study of Datopotamab Deruxtecan (Dato-DXd) in Combination With Rilvegostomig or Rilvegostomig Monotherapy Versus Pembrolizumab Monotherapy for the First-line Treatment of Participants With Locally-advanced or Metastatic Non-squamous NSCLC With High PD-L1 Expression (TC ≥ 50%) and Without Actionable Genomic Alterations (TROPION-Lung10)
Short Title:	Phase III, Open-label, Study of First-line Dato-DXd in Combination With Rilvegostomig for Advanced Non-squamous NSCLC With High PD-L1 Expression (TC ≥ 50%) and Without Actionable Genomic Alterations
Protocol Number:	304483877631871790
NCT Number:	NCT06357533
Sponsor Name:	AstraZeneca
Investigational Product:	pembrolizumab
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective: 1. Progression-Free Survival (PFS) in TROP2 biomarker positive participants.: PFS is defined as time from randomisation until progression per RECIST 1.1 as assessed by BICR, or death due to any cause.

The analysis will include all randomised participants, as randomised, regardless of whether the participant withdraws from randomised therapy, receives another anti-cancer therapy or clinically progresses prior to RECIST 1.1 progression, in the following population:

≥ TROP2 biomarker positive population

The measure of interest is the HR of PFS. PFS by investigator will be reported as a sensitivity analysis. 2. Overall Survival (OS) in TROP2 biomarker positive participants.: OS is defined as the time from randomisation until the date of death due to any cause.

The analysis will include all randomised participants, as randomised, regardless of whether the participant withdraws from randomised therapy or receives another anti-cancer therapy, in the following population:

≥ TROP2 biomarker positive population The measure of interest is the HR of OS.

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Secondary Obj.: 1. Progression-Free Survival (PFS) in the FAS population. 2. Overall Survival (OS) in the FAS population. 3. Assessment of Objective Response Rate (ORR) by BICR in TROP2 biomarker positive and FAS populations

2.2 Background & Rationale

The purpose of this study is to evaluate efficacy and safety of Dato-DXd in combination with rilvegostomig or rilvegostomig monotherapy compared with pembrolizumab monotherapy as a first line therapy in participants with locally advanced or metastatic non-squamous NSCLC with high PD-L1 expression (TC ≥ 50%) and without actionable genomic alterations.

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING

Target N: 675

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Histologically or cytologically documented non-squamous NSCLC.
- * Stage IIIB or IIIC or Stage IV metastatic NSCLC (according to Edition 8 of the AJCC staging manual) not amenable to curative surgery or definitive chemoradiation.
- * Absence of sensitising EGFR mutations, and ALK and ROS1 rearrangements, and absence of documented local test result for any other known genomic alteration for which there are locally approved and available targeted first-line therapies.
- * Must provide tumor sample to determine PD-L1 status, TROP2 status and other biomarkers.
- * Known tumour PD-L1 expression status defined as TC ≥ 50%
- * At least one lesion, not previously irradiated that qualifies as a RECIST 1.1 target lesion at baseline
- * ECOG performance status of 0 or 1
- * Adequate bone marrow reserve and organ function within 7 days before the first dose of study intervention

4.2 Exclusion Criteria

- * Prior systemic therapy for advanced/metastatic NSCLC.
- * Squamous cell histology, or predominantly squamous cell histology NSCLC; mixed small cell lung cancer; NSCLC histology, sarcomatoid variant.
- * History of another primary malignancy within 3 years
- * Active or prior documented autoimmune or inflammatory disorders (with exceptions)
- * Any evidence of severe or uncontrolled systemic diseases, including, but not limited to active bleeding diseases, active infection, active ILD/pneumonitis, cardiac disease.
- * Has clinically significant third-space fluid retention (for example pleural effusion) and is not amenable for repeated drainage.
- * History of non-infectious ILD/pneumonitis that required steroids, has current ILD/pneumonitis, or has suspected ILD/pneumonitis that cannot be ruled out by imaging at screening
- * Has significant pulmonary function compromise, as determined by the investigator

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- * Spinal cord compression, or brain metastases unless participant treated and no longer symptomatic, radiologically stable, and who require no treatment with corticosteroids or anticonvulsants.
- * History of leptomeningeal carcinomatosis
- * Known clinically significant corneal disease
- * Active infection with TB, HBV, HCV, Hepatitis A, or known HIV infection that is not well controlled
- * History of active primary immunodeficiency

11. Study Identifiers & Governance

Primary ID: 304483877631871790

Registry Number: NCT06357533

Sponsor: AstraZeneca

Study Title: Phase III, Open-label, Study of First-line Dato-DXd in Combination With Rilvegostomig for Advanced Non-squamous NSCLC With High PD-L1 Expression (TC ≥ 50%) and Without Actionable Genomic Alterations

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____